Nursing Essentials

Priority: First decide: expected pregnancy change, or a symptom that needs prompt assessment?

FOCUS 1

Assess

- **History:** gestational age, prior pregnancy outcomes, medical conditions, medications, allergies, and support needs.
- **Assess:** blood pressure, weight trend, symptoms, fetal status appropriate to gestational age, and ordered screening.
- **Teach:** prenatal visits, medication safety, nutrition, movement awareness, and how to reach the care team.

FOCUS 2

Comfort

- **Nausea:** small, frequent meals; fluids as tolerated. Persistent vomiting or dehydration needs evaluation.
- **Reflux:** smaller meals; remain upright after eating. Ask about pregnancy-appropriate treatment.
- **Constipation / backache:** fluids, fiber, activity as appropriate, posture support, and gentle body mechanics.

FOCUS 3

Escalate

- **Urgent:** vaginal bleeding, fluid leakage, severe abdominal pain, fainting, chest pain, or difficulty breathing.
- **Preeclampsia clues:** persistent severe headache, vision changes, or epigastric / right-upper-quadrant pain.
- **Also report:** fever, painful urination, unilateral leg swelling, or decreased fetal movement once established.

Assessment reminders

Make it stick: “Common in pregnancy” does not mean “ignore it.” Severity, associated symptoms, and change from baseline guide the response.

Focus	What to know	Nursing application
GTPAL	pregnancies; term births; preterm births; losses; living children. Twins count as one pregnancy, two living children.	Review the history
After midpregnancy	dizziness while supine may improve with lateral positioning; persistent symptoms need assessment.	Use safe comfort measures
Movement	teach the prescribed monitoring method. A meaningful decrease warrants contact, not waiting for the next visit.	Teach when to call

RED FLAGS · ESCALATE PROMPTLY

Bleeding, fluid leakage, severe pain, dyspnea, chest pain, fainting, or a meaningful decrease in fetal movement needs prompt assessment.